

Central City Health Center - Gastroenterology Department

123 Wellness Ave, Springfield, SP 54321 | Phone: (555) 123-4567 | Fax: (555) 765-4321

Dr. Alicia Green, MD, Gastroenterologist

Registration / Demographics

Patient Name	James L. Carter	Date of Birth	March 12, 1980
Age	43	Sex	Male
Address	789 Maple St, Springfield, SP 54321	Phone	(555) 867-5309
MRN	123456789	Account	987654321
Insurance	HealthPlan Choice Plus	Emergency Contact	Anna Carter, Wife, (555) 555-1234
Encounter Date	September 16, 2023	Check-in Time	08:30 AM
Location	CC Health Center, Endoscopy Suite 3	Attending Gastroenterologist	Dr. Alicia Green, MD
Referring Provider	Dr. Samuel Bridges, MD	Chief Complaint	Chronic dyspepsia and GERD evaluation
Past Medical History	GERD, Hypertension	Past Surgical History	Appendectomy (2005)
Family History	Colon cancer in father	Social History	Non-smoker, occasional alcohol
Allergies	Penicillin	Problem List	GERD, Hypertension
Current Medications	Omeprazole 20 mg daily, Lisinopril 10 mg daily, Metformin 500 mg BID	Pre-Procedure Vitals	BP: 128/76 mmHg, HR: 76 bpm, SpO2: 98%
ASA Classification	Class II	Informed Consent	Consent obtained. Patient understood and agreed to proceed.

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Pre-Procedure Assessment & Anesthesia Record

Pre-anesthesia Evaluation	Airway: No difficulties anticipated. Cardio/Pulmonary: Clear lung sounds, regular heart rhythm. NPO since midnight. Planned Sedation: Moderate sedation with Propofol and Fentanyl planned. No prior adverse anesthesia reactions noted.		
Anesthesia Team	Dr. Jonathan Lee, Anesthesiologist; Emily Tran, CRNA		
Anesthesia Start	09:00 AM	Scope In	09:15 AM
Scope Out	09:45 AM	Anesthesia End	10:00 AM
Recovery Start	10:05 AM	Recovery End	10:45 AM

Monitoring Trends

Time	BP	HR	SpO2
09:00 AM	130/82	78	98%
09:20 AM	125/78	75	97%
09:40 AM	124/76	74	98%
10:00 AM	128/80	76	98%

Complications: Brief drop in SpO2 to 92% at 09:25 AM, resolved with a jaw thrust and oxygen increase. No further issues observed.

Post-anesthesia Evaluation: Patient met all criteria with an Aldrete score of 10. Discharged home with escort at 10:50 AM.

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Procedure Note: Esophagogastroduodenoscopy (EGD)

Endoscope Model: EndoView 3000. Patient was placed in the left lateral decubitus position with a bite block for airway protection. The endoscope was introduced smoothly into the oropharynx.

Upon entering the esophagus, the mucosal surface appeared normal with no varices observed. The Z-line was clearly demarcated at 40 cm from the incisors. The gastroesophageal junction (GEJ) appeared with no stigmata of bleeding or erosion.

Transitioning into the stomach, the fundus and body were systematically inspected. A moderate degree of erythema was noted along the antrum with no ulcerations. Good insufflation demonstrated intact rugal folds. The pylorus appeared patent.

In the duodenum, the bulb and the second portion appeared normal, with visible villi and no evidence of ulceration or inflammation.

Retroflexion in the stomach allowed for a clear assessment of the cardia and fundus with no abnormalities noted.

Findings:

- Esophagus: Normal Z-line at 40 cm, no signs of esophagitis.
- Stomach: Moderate erythema in the antrum.
- Duodenum: Normal appearance.

Impression:

This is a case of chronic dyspepsia with findings suggestive of gastritis without active ulceration.

Diagnosis:

Chronic gastritis; GERD

Interventions:

No biopsies were taken given the absence of suspicious lesions, and there were no interventions required.

Endoscopy Team: Dr. Alicia Green, MD; John Smith, RN (Circulator); Karen White (Endoscopy Tech)

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Specimens / Lab / Pathology

No specimens were obtained during this procedure as there were no findings warranting biopsy or other sampling.

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After Visit Summary / Discharge Instructions

Recovery Course: Patient was alert and oriented. No pain or nausea reported. Tolerated oral intake without any issues, and vitals remained stable throughout recovery.

Instructions:

Diet: Resume regular diet as tolerated. Avoid spicy and fried foods for 24 hours.

Activity: Rest for the remainder of the day. Resume normal activities as tolerated the following day, avoiding heavy lifting for 48 hours.

Medications: Continue Omeprazole 20 mg daily. Avoid NSAIDs. Continue other medications as directed.

Warning Signs: Contact provider if experiencing severe abdominal pain, vomiting blood, or black tarry stools.

Follow-up: Schedule a follow-up appointment in 4 weeks with your primary care physician or gastroenterologist. Reach out if symptoms persist or worsen.

Results Communication:

Results will be communicated through MyHealth Portal or by phone. For any questions or concerns, please call the office at the number provided.

Patient confirmed understanding of all discharge instructions and verbalized comprehension of follow-up plan. Transported home by wife.

Discharge Educator: Sarah L. Brown, RN

Date: September 16, 2023 | Time: 10:50 AM

Electronically signed by Dr. Alicia Green, MD on September 16, 2023 at 11:00 AM

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